

Major Depressive Disorder - Overview

DIAGNOSTIC CRITERIA (DSM-5):

- 5 or more symptoms for at least 2 weeks
- At least one: depressed mood OR anhedonia
- Insomnia or hypersomnia, fatigue, poor concentration
- Psychomotor agitation or retardation
- Feelings of worthlessness or excessive guilt
- Recurrent thoughts of death, suicidal ideation

MNEMONIC: SIG E CAPS

Sleep, Interest, Guilt, Energy, Concentration, Appetite, Psychomotor, Suicidality

EPIDEMIOLOGY:

- Lifetime prevalence 15-20%; women 2x more than men
- Peak onset 20-30 years old
- Recurrence: 50% after first episode, 70% after second

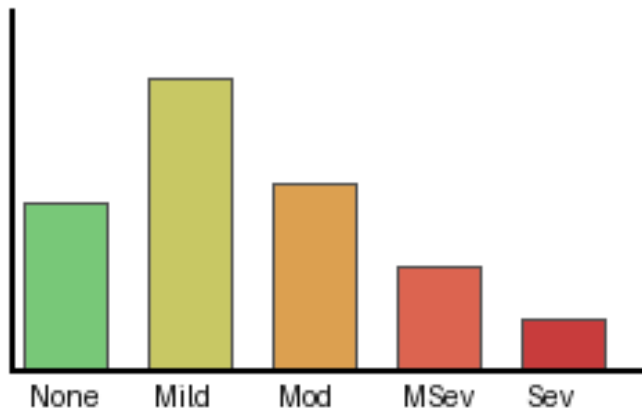
Screening and Severity Assessment

SCREENING TOOLS:

- PHQ-9: 0-27 scale; score ≥ 10 likely MDD (sens 88%, spec 88%)
- BDI (Beck Depression Inventory): 21-item self-report

SCORE DISTRIBUTION - BDI (chart below):

BDI Score Distribution



- PHQ-2 (first 2 items) is rapid screener; if ≥ 3 proceed to full PHQ-9
- Always assess item 9 (suicidal ideation) regardless of total score

Treatment of Major Depressive Disorder

FIRST-LINE TREATMENT:

- Moderate-severe MDD: pharmacotherapy + psychotherapy
- Mild MDD: psychotherapy alone may suffice

ANTIDEPRESSANT COMPARISON:

Drug	Class	Key Side Effects	Special Notes
Sertraline	SSRI	GI upset, sexual dysfunction	First-line; cardiac safe
Escitalopram	SSRI	QTc prolongation (high dose)	Best tolerability
Bupropion	NDRI	Seizure risk, insomnia	No sexual SE
Venlafaxine	SNRI	HTN, diaphoresis	Also for anxiety/pain
Mirtazapine	NaSSA	Sedation, weight gain	Helpful in insomnia

PSYCHOTHERAPY OPTIONS:

- CBT: most evidence; reframes negative cognitions
- IPT: interpersonal role transitions and grief

REFRACTORY MDD:

- ECT: most effective for severe/refractory cases
- Augment: lithium, aripiprazole, quetiapine